

URGENT SUSPECTED CANCER REFERRAL

ROYAL STOKE UNIVERSITY HOSPITAL

General Enquiries:

Tel 01782 671117

COLORECTAL

ALL REFERRALS TO BE SENT VIA THE ELECTRONIC REFERRAL SYSTEM (ERS)

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, updated October 2023 and DG56 August 2023

PATIENT DETAILS				GP DETAILS			
Name:	Given Name Surname			Name:	Registered GP Full Name		
Address:	Home Full Address (single line)			Address:	Organisation Name, Organisation Full Address (single line)		
NHS Number:	NHS Number			Phone No:	Organisation Telephone Number		
Hospital Number:	Hospital Number			E Mail:	Organisation E-mail Address		
Date of Birth:	Date of Birth			Name of referrer:			
Sex:	Gender(full)			Decision to refer date:			
Interpreter required:	☐ Yes ☐ No			Sign Language required:	☐ Yes ☐ No		
Language:	Main Language						
Contact No (next 48 hrs):	Home:	Patient Home Telephone	Work:	Patient Work Telephone	Mobile:	Patient Mobile Telephone	

Information provided will be used for Triage to Test.
Incomplete or incorrect information could result in incorrect test being requested and a delay in diagnosis

REASON FOR REFERRAL (please indicate as appropriate)

Any Age:

- ☐ Unexplained anal mass or anal ulceration (FIT is also recommended)
- ☐ Rectal Mass
- ☐ Suspicion of colorectal malignancy on imaging

If any of the reasons for referral below are selected the result of a FIT is also required which must have been reported in the previous 3 months

(ABNORMAL FIT >10) enter result here: Single Code Entry: Quantitative faecal immunochemical test

☐ Any age with an Abdominal mass

☐ Aged 40 or over with unexplained weight loss and abdominal pain

Aged 50 years and over with any of the following unexplained symptoms

- ☐ Rectal Bleeding which is either ☐ Fresh Rectal Blood or ☐ Dark Rectal Blood
- ☐ Abdominal pain ☐ Weight loss

Aged Under 50 Years with Rectal bleeding (please indicate if this is fresh or dark)

- ☐ Fresh Rectal Blood ☐ Dark Rectal Blood

AND either of the following unexplained symptoms

- ☐ Abdominal Pain ☐ Weight loss

☐ Iron Deficiency Anaemia Aged 60 Years and over

☐ Change in Bowel Habits

A **digital rectal examination** is required to be done as this ensures any anal pathology is noted and that patients are then allocated to the most appropriate direct test. If a digital rectal examination is not possible please indicate reasons below.

Please state findings of DRE

Any other relevant symptoms not covered by the guidelines

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Is the patient on anticoagulants ☐ Yes ☐ No If Yes please give details of these drugs	Is the patient on Antiplatelet Drugs ☐ Yes ☐ No
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Accessibility Needs: ☐ Wheelchair access ☐ Deaf ☐ Registered blind ☐ Learning Disability ☐ Other disability needing consideration ☐ Accompanied by carer	WHO Performance Status: ☐ 0 Fully active ☐ 1 Able to carry out light work ☐ 2 Up and about greater than 50% of waking time ☐ 3 Confined to bed/chair for greater than 50% ☐ 4 Confined to bed/chair 100%
RISKS: ☐ Vulnerable Adult (detail below if any recording within last 3 years) ☐ Patient does not have capacity to consent Any other known risk:	

- ☐ I confirm that I have discussed the possibility that the diagnosis may be cancer and they may wish to take a family member or significant other to the initial appointment.
- ☐ Patient is fully aware that investigations may result in additional significant findings that require further direct referral to other specialities
- ☐ Patient agrees to have an invasive test (Endoscopy/CT Colonogram/CT Thorax, Abdomen and Pelvis) and is available to attend for the investigations
- ☐ I confirm that recent (reported in the last 90 days)
- FBC, U+E, eGFR and CRP results are available (if blood tests not requested from UHNM via ICE, please attach results)
- In addition
- Serum Ferritin and tTG IgA (if being referred for Iron Deficiency Anaemia).
 - Coagulation profile will be required if patient is on anticoagulants. *(not required if on Direct Oral Anticoagulants (DOAC) or Low molecular weight heparin (LMWH))*
- ☐ I confirm that the patient is available to attend for further investigation within the next 14 days

If patient is not available for the next 2 weeks, and aware of nature of referral, consider deferring the referral and seeing the patient again to reassess symptoms

Smoking Status:
Smoking

Alcohol Consumption
Alcohol Consumption

Allergies:
Allergies

Medication:
Medication

Relevant medical history:
Problems

Investigations
Investigations

Name: Given Name Date of Birth NHS No: NHS Number
 UHNM - Colorectal Urgent Suspected Cancer Referral Proforma
 V7 UHNM (July 2024)

NICE DG56 Guidance

Quantitative faecal immunochemical testing (FIT) using HM-JACKarc or OC-Sensor is recommended to guide referral for suspected colorectal cancer in adults:

- with an abdominal mass, or
- with a change in bowel habit, or
- with iron-deficiency anaemia, or
- aged 40 and over with unexplained weight loss and abdominal pain, or
- aged under 50 with rectal bleeding and either of the following unexplained symptoms:
 - abdominal pain
 - weight loss, or
- aged 50 and over with any of the following unexplained symptoms:
 - rectal bleeding
 - abdominal pain
 - weight loss, or
- aged 60 and over with anaemia even in the absence of iron deficiency.

FIT should be offered even if the person has previously had a negative FIT result through the [NHS bowel cancer screening programme](#). People with a rectal mass, an unexplained anal mass or unexplained anal ulceration do not need to be offered FIT before referral is considered.

